

Patient ID: S (Chair)Period: January 11 to June 30, 2024Report Date: July 1, 2024Coverage: 3525/4110h (82.7%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 172 days from Jan 11 to Jun 30

■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
18	42h	High Heart Rate	<1	86%

Major Findings: Sustained high HR (avg 89, peak 109)

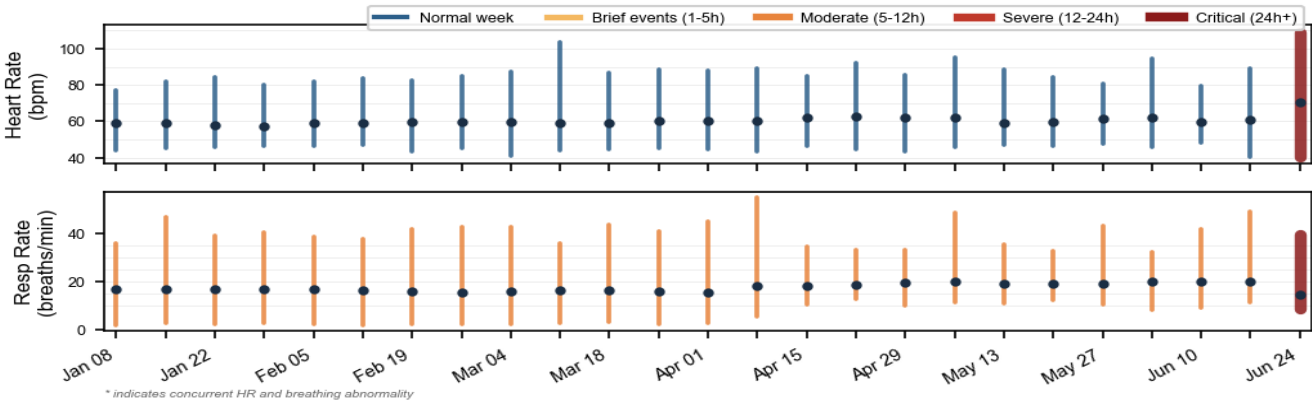
#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	High Heart Rate	109 bpm	7h	7h	1	89 bpm	Jun 27
2	Low Heart Rate	40 bpm	4h	3h	1	52 bpm	Jun 24 to Jun 25
3	Elevated Heart Rate	106 bpm	15h	3h	3	88 bpm	Jun 26 to Jun 28
4	Elevated Breathing	44 brpm	13h	10h	1	23 brpm	Jun 21 to Jun 24
5	Elevated Breathing	26 brpm	1h	1h	1	16 brpm	Apr 03
6	Elevated Breathing	30 brpm	1h	1h	1	20 brpm	Apr 10

+ 1 additional condition(s) across Elevated Breathing; details in monitoring data.

Daily recorded hours declined from 8.0 to 3.0 (62%) over the monitoring period. RR values outside physiologic range (6–60 breaths/min).

- Clinical Pattern Observations:
- 1. Sustained finding: 10h continuous Elevated Breathing on Jun 24.
 - 2. Clustered pattern: Episodic events on 4 of 172 days (2%).
 - 3. Nocturnal pattern: 6 of 6 eligible episodes during evening or night hours.

Weekly Aggregated Trends (25 weeks)



Weekly bars; color = episode burden. Badges = episodic hours.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm

Low HR < 45 bpm

Elevated HR > 95 bpm

High HR > 100 bpm
- Very High HR > 110 bpm

Elevated Breathing > 24 brpm

High Breathing > 30 brpm

Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.